

กิจกรรมแลกเปลี่ยนเรียนรู้ ชื่อเรื่อง : Gender-Based Violence and Reproductive Coercion

อาจารย์ ดร. ศศิธรา น่วมภา วิทยาการ

Although GBV is the most pervasive human rights violation in the world, it is the least understood. GBV includes psychological (47.1%), sexual (8.8% rape and 15.8% other), and physical (31.5%) violence. One third of women age 18 and older experience intimate partner violence (IPV), with 70% first experiencing violence prior to age 25; a period in the life course when women have the highest rates of unintended pregnancy and sexually transmitted infections. Women in violent relationships are at greater risk of unintended pregnancy due in part to reproductive coercion (RC). Reproductive coercion includes pregnancy coercion- a male partner's attempts to pressure a female partner to become pregnant (19%) or to end a pregnancy, and contraceptive sabotage (15%)-a male partner's interfere with contraception. Fear of violence if they use contraception or request condom use has been shown to reduce women's likelihood of using contraception. GBV has long-term implications for mental, physical, and reproductive health, making GBV a global public health challenges, as well as a barrier to civic, social, political, and economic participation. It undermines the safety, dignity, overall health status, and human rights of the millions of individuals who experience it, as well as the public health, economic stability, and security of nations. GBV and RC occur across geographic and cultural contexts, causing acute and chronic physical, psychological, and reproductive injury. One third of adolescents report their first sexual encounter to be forced.

Past studies have found associations between IPV and gynecological disorders (e.g. vaginal discharge, sexually transmitted infections, unplanned pregnancies), adverse pregnancy outcomes (e.g., low birth weight, preterm birth, pregnancy loss), chronic pain (e.g., pain, migraines, fibromyalgia), and mental health complications (e.g., depression, anxiety, post-traumatic stress disorder, substance abuse, suicidality). There is mounting evidence that trauma across the lifespan, such as that caused by GBV, contributes to chronic disease, premature ageing, and ultimately increases morbidity, and premature mortality. Therefore,

the outcomes of understanding reproductive coercion and GBV can reduce negative reproductive health and mental health.

